

February 23, 2026

Jim O'Neill
Deputy Secretary
US Department of Health and Human Services
200 Independence Avenue SW Washington, DC 20201

Dr. Thomas Keane
Assistant Secretary and National Coordinator
Assistant Secretary for Technology Policy /
Office of the National Coordinator for Health Information Technology
330 C Street NW Floor 7, Mary E. Switzer Building Washington, DC 20201

Submitted electronically to <http://www.regulations.gov>

RE: HHS Sector AI RFI (2025-23641) (90 FR 60108)

Dear Deputy Secretary O'Neill and Assistant Secretary Keane:

[PointClickCare \(PCC\)](#) appreciates the opportunity to comment on the Office of the Deputy Secretary and Assistant Secretary for Technology Policy / Office of the National Coordinator for Health Information Technology (ASTP/ONC) Request for Information: [Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care \(HHS Health Sector AI RFI\)](#), as published in the December 23, 2025 Federal Register (RIN 0955-1113).

PCC is a leading health technology company with one mission: to help providers deliver exceptional care. For over 20 years, we have been transforming how care is planned, delivered, and paid for across the long-term and post-acute care (LTPAC) continuum. PCC has (2) two Certified EHR Technology (CEHRT) systems and supports one of the largest LTPAC datasets in the United States, powering AI-informed care transitions, clinical outcomes, and reimbursement workflows. We are also setting a new standard in interoperability as the only post-acute-focused organization participating in all major federal interoperability initiatives, including ONC certification, TEFCA connectivity through partners, and standards-based FHIR exchange.

More than 27,000 LTPAC organizations, 3,600 ambulatory clinics, 2,800 hospitals, 350 risk-bearing providers, 70 state and government agencies, and every major U.S. health plan trust PCC to help them navigate a changing healthcare landscape. Our Marketplace of over 400 integration partners enables customers to connect care, billing, and operations, driving efficiency and better outcomes. At PCC, we are committed to building a stronger, more resilient healthcare system that fully includes older adults, people with disabilities, and safety-net populations served in post-acute and senior care settings.

Executive Summary and Key Recommendations

We commend HHS for launching a “OneHHS” strategy on AI that explicitly links regulation, reimbursement, and research & development, and for seeking experience-based feedback from those building and deploying AI in clinical care. From our vantage point as a leading post-acute EHR and interoperability platform, we see AI’s potential to improve care transitions, reduce avoidable hospitalizations, and alleviate workforce burden, particularly in LTPAC and Medicaid-serving settings. To realize that potential and support HHS’s three major levers, we recommend HHS prioritize five cross-cutting actions:

1. Clarify regulatory expectations for non-device AI in clinical workflows

Establish a unified, risk-proportionate framework for AI used in triage, summarization, documentation assistance, risk stratification, and other non-device decision support. This should include clear guidance on risk categories, clinician oversight, liability allocation, documentation and audit expectations, and permissible development and deployment practices under HIPAA and other HHS authorities, aligned with ONC’s decision support transparency concepts.

2. Align reimbursement with measurable outcomes and burden reduction

Modernize payment policy so that providers deploying validated AI tools that demonstrably improve outcomes, reduce avoidable utilization, or reduce documentation burden can share in the value created. This could include add-on payments, shared-savings recognition, or quality measure credit within Medicare fee-for-service and value-based programs, including CMMI demonstrations targeted to post-acute and senior care.

3. Support a real-world AI evaluation “playbook” and shared benchmarks

Promote standardized, human-centered evaluation methods, including safety, drift, bias, and workflow impact tailored to LTPAC, SNF, and senior living environments. HHS can seed evaluation toolkits, shared reference datasets, and benchmark measures through grants, cooperative agreements, and public–private testbeds that smaller providers and vendors can use to assess AI tools consistently.

4. Advance pragmatic interoperability and data quality for AI readiness

Prioritize USCDI and USCDI+ data elements and FHIR-based exchanges most relevant to post-acute and senior care (e.g., real-time ADT, medication, functional and cognitive status, and social risk data) and ensure LTPAC providers are included in FHIR API and TEFCA implementation efforts. Pair this with minimum data quality expectations and clear guidance on data provenance so AI models have reliable inputs across care settings.

5. *Invest in post-acute-focused AI research and multi-site demonstration projects*

Through CMMI models, NIH and AHRQ partnerships support testbeds that evaluate AI's impact on falls, readmissions, pressure injuries, medication safety, behavioral health, and care transitions across acute–post-acute boundaries. These efforts should embed equity, bias mitigation, and small-provider readiness into their design and include LTPAC providers as full partners.

We strongly support HHS's goals to foster public trust and confidence in modern technology solutions, reduce uncertainty that impedes AI innovation, and align federal incentives so that AI enhances productivity, reduces burden, lowers cost, and improves health outcomes for patients, caregivers, and communities. The detailed responses below provide specific recommendations aligned with each RFI question.

Why Post-Acute AI Policy Requires Distinct Consideration

Digital and interoperability starting point

Post-acute and long-term care providers were largely excluded from the federal HITECH incentive framework that accelerated EHR adoption in hospitals and physician practices. As a result, skilled nursing facilities (SNFs), assisted living communities, and other LTPAC providers entered the modern interoperability era without the same level of federal capital support, technical assistance, or infrastructure standardization. Many post-acute organizations have since achieved significant digital maturity, yet interoperability gaps, particularly around real-time ADT notifications, structured laboratory feeds, and standardized functional and cognitive status data, remain more pronounced than in acute care.

AI policy that assumes uniform digital infrastructure across settings risks unintentionally reinforcing these disparities. To meet the goals of the Health Sector AI RFI, a “OneHHS AI” strategy must explicitly account for the distinct starting point of LTPAC providers and ensure that data standards, certification pathways, payment models, and technical assistance all include post-acute environments.

Financing and workforce realities

Financing and workforce realities further differentiate post-acute care from other sectors. Medicaid is the dominant payer for long-term care services, and reimbursement rates often operate on thin margins that limit flexibility for capital investment. Unlike many hospital systems, most SNFs and senior care providers are not positioned to absorb significant upfront technology costs without clear, aligned incentives.

The workforce structure in post-acute settings, characterized by interdisciplinary teams led by medical directors and heavily supported by licensed practical nurses, certified nursing assistants, and care coordinators, shapes how AI must be designed and governed. Tools that assume constant physician presence, always-on specialist coverage, or large internal analytics teams may not translate effectively into SNF environments. AI policy must therefore reflect staffing composition, scope-of-practice

considerations, and survey oversight dynamics unique to post-acute care to ensure that innovation enhances, rather than destabilizes, care delivery for older adults and individuals with disabilities.

We encourage HHS to view post-acute AI not as a downstream extension of hospital AI, but as a distinct policy domain with tailored incentives, measurement strategies, and technical assistance to avoid widening existing gaps in quality and equity.

Responses to Specific Questions

1. Biggest barriers to private sector innovation in AI for health care

In summary, the biggest barriers we observe in post-acute and senior care are: (1) regulatory ambiguity for non-device AI, (2) misaligned payment incentives, (3) fragmented interoperability and state-level variation, and (4) capital and workforce constraints that limit the ability of LTPAC providers to evaluate and govern AI tools.

First, regulatory uncertainty surrounding non-device AI tools creates hesitation among providers. Many AI-enabled tools, such as readmission prediction models, documentation assistance systems, and risk stratification tools, do not meet the definition of regulated medical devices but meaningfully influence clinical workflows and reimbursement. Providers remain uncertain about how FDA, ONC, CMS, and state regulators will interpret the use of these tools in audits, surveys, and enforcement contexts. In risk-averse post-acute environments, ambiguity leads to defensive non-adoption.

Second, payment incentives are frequently misaligned. Under fee-for-service Medicare, providers bear the costs of AI implementation while savings from avoided hospitalizations or improved coordination often accrue primarily to payers. Although AI adoption is more financially rational in capitated or shared-savings models (such as certain risk-bearing entities or CMMI arrangements), many post-acute providers remain outside such arrangements.

Third, fragmented interoperability and incomplete data flows constrain both model development and deployment. Post-acute providers were historically excluded from earlier federal health IT incentive programs, and access to structured laboratory data, medication feeds, and real-time ADT notifications remains inconsistent. AI systems depend on reliable, cross-setting data inputs; underrepresentation of LTPAC and dual-eligible populations in training datasets also limits generalizability.

Fourth, state-level fragmentation presents an additional barrier. Because post-acute care is heavily regulated by state survey agencies and Medicaid programs, variation in privacy interpretations, documentation standards, and oversight practices complicates multi-state AI deployment. Federal coordination and model guidance could reduce this variability while preserving state authority.

Finally, capital and workforce constraints are particularly acute in SNF and senior care environments. Many facilities operate on thin margins and lack internal data science resources to independently evaluate model performance, assess bias, or monitor drift. Without structured evaluation support, providers may either avoid adoption or deploy tools without sufficient governance.

2. Regulatory, payment policy, or programmatic design changes HHS should prioritize

HHS should prioritize development of a unified, risk-proportionate framework for non-device AI used in clinical workflows. This framework should clarify expectations for clinician oversight, documentation integrity, transparency when AI assists in drafting clinical notes or generating predictive insights, and permissible training data practices under HIPAA. It should also reinforce expectations for bias testing and equity impact assessment consistent with civil rights obligations and align with ONC's emerging transparency requirements for decision support interventions.

Rather than broad liability protections, HHS could consider structured accountability pathways when validated AI tools are deployed within defined governance frameworks and with documented clinician review. For example, guidance could clarify that AI-assisted documentation where clinicians retain final review and attestation does not by itself constitute fraudulent intent, while still requiring appropriate monitoring and audit trails. Predictable oversight encourages responsible innovation without weakening safeguards.

On the payment side, CMS should explore mechanisms that recognize validated AI-enabled interventions within value-based programs such as the SNF Value-Based Purchasing Program and related Quality Reporting Programs. Transitional, time-limited innovation add-on payments, shared-savings recognition, or quality measure credit may help ensure adoption is not limited to large risk-bearing entities. Any payment recognition should be tied to measurable outcome improvement, such as reduced avoidable hospitalizations or improved functional outcomes and include guardrails to prevent inappropriate coding or utilization changes.

Cross-agency coordination among ONC, CMS, FDA, and OCR would further reduce conflicting signals and streamline expectations for developers and providers. A coordinated "AI in Clinical Workflows" council or task force could align regulatory guidance, transparency expectations, and payment models under the broader HHS AI Strategy.

3. Novel legal and implementation issues for non-medical device AI and HHS's role

Non-device AI raises complex questions regarding responsibility allocation between developers and providers, documentation expectations, audit discoverability, and contractual risk allocation. Clarification is needed regarding how clinician review should be documented when AI assists in drafting content, and how providers should approach governance and monitoring responsibilities over time.

HHS can support responsible implementation by publishing model governance frameworks and encouraging transparency in data stewardship practices, including disclosure of secondary model training uses. Providers should understand how their data may be used to refine or commercialize models and what safeguards apply.

AI-specific cybersecurity risks also warrant attention. Model manipulation, prompt injection, and third-party dependency vulnerabilities represent evolving risks that should be incorporated into existing health sector cybersecurity guidance and incident response expectations.

Implementation challenges are particularly acute for smaller and independent SNFs that lack in-house data science or advanced IT teams. Many facilities do not have the capacity to independently validate models, audit for bias, or design governance processes. Standardized evaluation templates, shared reference datasets, and technical assistance models tailored to LTPAC settings would help ensure consistent safeguards and reduce “defensive non-adoption.”

4. Evaluation methods, metrics, robustness testing, and human-centered evaluation

Evaluation should scale to risk: assistive documentation tools require different oversight than predictive triage systems or deterioration alerts that may influence clinical intervention. Outside of the medical device context, there is currently no widely adopted U.S. standard for risk categorization of non-device clinical AI tools, even as other jurisdictions develop risk-based governance approaches. HHS could promote consistency by articulating a clear risk taxonomy for non-device AI used in clinical care, such as high-, medium-, and low-impact categories, to proportionally scale evaluation depth, documentation expectations, and post-deployment monitoring requirements.

In doing so, HHS may wish to consider established international risk frameworks, including the impact-based approach by the International Medical Device Regulators Forum (IMDRF) in its Software as a Medical (SaMD) guidance, which stratifies oversight based on the significance of information provided and the state of the healthcare situation. While non-device AI tools may fall outside formal SaMD scope, the IMDRF model provides a structured way to assess clinical impact and could serve as a conceptual reference point. Other quantitative and qualitative approaches, such as impact scoring based on workflow dependency, automation level, or potential oversight model. Referencing an established risk framework would promote clarity and alignment without creating unnecessary duplicative regulatory structure.

Because many post-acute providers lack internal analytic resources, HHS could fund development of standardized evaluation playbooks, metric sets, and benchmarking tools tailored to LTPAC environments through grants, cooperative agreements, and prize competitions. These toolkits would help small SNF operators interpret performance metrics, assess impact categories, ask more informed questions of vendors, and frame governance decisions around safety, bias, and reliability.

5. Supporting private sector accreditation, certification, and credentialing

Expansion of voluntary certification frameworks addressing AI transparency, explainability, risk management, and monitoring would promote consistency across vendors. The ONC Health IT Certification Program already incorporates required transparency criteria for decision support interventions in certified EHR technology; this framework provides a useful foundation that could be extended or adapted to address emerging AI-specific governance considerations such as model monitoring, drift management, and documentation of training data provenance. Coordination with accrediting bodies could further integrate AI governance readiness into organizational standards so that oversight expectations are aligned across technology developers and provider organizations.

Additionally, clinician AI literacy should become part of continuing education expectations. Licensed professionals must understand model limitations, potential bias, appropriate override practices, and documentation responsibilities. HHS should work with licensing boards and professional societies to integrate core AI literacy into continuing education and competency frameworks so that AI augments, not replaces, licensed staffing requirements or professional judgment.

6. Where AI has met or fallen short; high-potential tools

AI has demonstrated promise in readmission risk prediction, documentation summarization, and medication reconciliation when embedded directly into workflows. In high-volume SNF environments, documentation summarization tools can reduce chart review time and allow reallocation of nursing attention to resident care.

However, predictive tools that lack explainability or workflow integration often fail to produce measurable impact. Standalone dashboards or alerts that are not aligned with existing team processes or care pathways have limited real-world value and can contribute to alert fatigue.

High-potential areas for post-acute and senior care include deterioration prediction, behavioral health and cognitive status monitoring, automated quality measure abstraction, and AI-assisted patient-to-setting matching across acute and post-acute transitions. HHS can help ensure that evidence from these use cases is captured and linked to payment and regulatory policies that reward proven value.

7. Decision makers, governing bodies, and administrative hurdles

In SNF and senior care settings, adoption decisions for AI-supported tools are typically led by medical directors, chief or director-level nursing leaders, and IT leaders (such as a VP of IT or CIO), with compliance, privacy, and finance leaders playing critical review roles. Governance bodies such as QAPI committees often play a central role in evaluating and monitoring AI tools once deployed.

The primary administrative hurdles are the cross-functional coordination required security and privacy reviews, legal and contracting cycles, clinician training, workflow redesign, and change management, layered on top of capital constraints and uncertainty about how regulators and surveyors will interpret AI-supported documentation and workflows. Model contracting templates, governance guidance, and clear survey expectations would reduce friction and promote consistent safeguards.

8. Interoperability to widen market opportunities, fuel research, and accelerate AI

AI scalability depends on improved interoperability across the care continuum. Real-time ADT feeds, structured medication and laboratory data, harmonized functional and cognitive assessments and standardized social needs data are critical inputs for predictive and care coordination tools. When these data elements are incomplete or delayed at transitions between hospitals and post-acute care settings, AI models lose reliability and care coordination suffers.

Expansion of USCDI and USCDI+ to reflect post-acute and senior care data elements will strengthen model accuracy and ensure inclusion of Medicaid-serving populations. Inclusion of LTPAC providers

and their vendors in FHIR-based API initiatives and TEFCA implementation is essential to ensure cross-setting data completeness. We encourage HHS to ensure that TEFCA exchange purposes, FHIR API policies, and USCDI expansion explicitly include LTPAC-relevant use cases so that cross-setting AI tools do not “break” at the post-acute boundary.

In addition to transactional interoperability, HHS could strengthen AI readiness through development of controlled-access, de-identified reference datasets designed specifically for research, validation, and benchmarking in post-acute and safety-net populations. These datasets should be structured to support model evaluation and fairness testing, rather than commercial product replication, and should include longitudinal cross-setting indicators such as Admission/Discharge/Transfer (ADT) events, standardized assessment variables, medication categories, and outcome measures (e.g., rehospitalization, falls, pressure injuries) with consistent documentation of data provenance and limitations.

Conceptually, this approach could resemble CMS Limited Data Set (LDS) files in its use of HIPAA-compliant de-identification and governed data use agreements, but be purpose-built for AI validation in LTPAC populations. Access should be tiered, subject to appropriate data use agreements, and focused on benchmarking and methodological transparency rather than real-time operational deployment. Such infrastructure would help smaller providers, researchers, and emerging vendors assess subgroup performance, compare outcomes across representative populations, and strengthen model accountability, while preserving the proprietary, workflow-integrated datasets that drive real-world implementation and care delivery.

By clearly distinguishing benchmarking infrastructure from operational data exchange, HHS can promote equity, reduce duplication of effort, and lower barriers to responsible AI development without disrupting the competitive and innovation dynamics that enable vendors and providers to invest in advanced data integration and care transformation.

9. Patient and caregiver priorities and concerns

Patients and caregivers in post-acute and senior care settings prioritize fewer avoidable hospitalizations, earlier detection of clinical decline, improved medication safety, and more clinician time at the bedside. They want AI to help coordinate care across facilities and providers, reduce paperwork, and make it easier to navigate complex care plans.

Concerns focus on privacy and security, transparency, potential algorithmic bias, and overreliance on automation. There is legitimate worry that AI systems might treat older adults, people with disabilities, or individuals with complex social needs unfairly or opaquely.

HHS can promote trust by encouraging plain-language AI disclosures and embedding patient and caregiver perspectives into evaluation and oversight frameworks—for example, through advisory panels, user testing, or public comment on key guidance. Embedding these perspectives will help ensure that AI tools respond to real-world concerns about dignity, autonomy, and safety.

10. Priority AI research areas and literature considerations

HHS should prioritize research in LTPAC populations focusing on functional outcomes, falls, readmissions, pressure injuries, medication-related harm, and implementation science in resource-constrained environments. Research should also examine how AI can support workforce augmentation and burden reduction without reducing necessary staffing or eroding human relationships in care.

Emerging literature suggests that AI tools can reduce readmissions, improve early detection of deterioration, and increase documentation efficiency when properly validated and embedded in workflows, but that performance often degrades when models are deployed outside the populations or settings in which they were trained. This reinforces the need for local validation, fairness testing, and ongoing monitoring, particularly in Medicaid and dual-eligible populations.

These research priorities can be advanced through targeted NIH and AHRQ funding calls and CMMI demonstration models that include post-acute and Medicaid-serving providers as full partners, not just downstream data sources.

Conclusion

Artificial intelligence can strengthen quality, reduce burden, and support value-based care across the continuum. In post-acute and senior care settings, success will depend on regulatory clarity and risk-proportionate oversight, payment alignment that rewards measurable improvements, interoperability inclusion and data quality, standardized evaluation support, transparent data stewardship, and strong governance that maintains human oversight and protects equity.

PCC welcomes continued engagement with HHS to ensure AI improves care for older adults, people with disabilities, and Medicaid-serving populations in ways that are safe, equitable, and accountable. We stand ready to partner with HHS and its agencies through pilots, testbeds, and advisory efforts to help make the HHS AI Strategy and Health Sector AI RFI goals a reality.

Sincerely,

Dr. Eugene Gonsiorek, PhD
VP Clinical Regulatory Standards
PointClickCare